**This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.**

Patient Group Direction

# for the administration of travel vaccines to children and young people (Junior Travel Vaccines)

## Hepatitis A, Hepatitis A and B, Typhoid, Meningococcal ACWY, Rabies, Japanese Encephalitis and Cholera

Patient Group Direction, version 009, issued 30 September 2026. Supersedes Version 008, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

## Guideline Summary: pre-travel assessment in children and young people

- Every child travelling should have a pre-travel risk assessment covering destination, duration, season, style of travel, rural or urban itinerary, planned activities, animal contact, access to medical care, and any underlying health conditions.
- Check and complete the routine UK childhood immunisation schedule first. Many travel-related infections, including diphtheria, tetanus, polio, measles and meningococcal disease, are covered by routine vaccines, and catch-up is a priority before travel-specific vaccines are considered.
- Vaccine choice and dose in children are age-dependent. Several travel vaccines have paediatric presentations with a lower antigen content or a different schedule, and some have a minimum age below which they are not licensed or not immunogenic.
- Allow adequate time before departure for courses to be completed. Where time is short, accelerated schedules may be available for some vaccines but must be checked against the SPC and current national guidance.
- Malaria chemoprophylaxis, insect bite avoidance, food and water hygiene, road traffic safety and rabies avoidance advice are essential parts of the consultation and are not replaced by vaccination.
- Country-specific recommendations change. TravelHealthPro (NaTHNaC) must be checked at the time of each consultation.

**Live vaccines and immunosuppression**

- Children who are significantly immunosuppressed, including those on high-dose corticosteroids or biologic therapy, and children with asplenia or complement disorders, require specialist advice and are excluded from this PGD.
- Where a live vaccine is indicated and the child has received another live vaccine within the previous 4 weeks, seek advice on timing.

# Patient Group Direction

for the administration of

**Junior Travel Vaccines**

for the prevention of travel-related infection in children and young people

## Healthcare professionals covered and training

| | Healthcare professionals allowed to work under this PGD and requirements |
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| Qualifications and Registration | ● Pharmacist registered and practicing with the GPhC<br>● Pharmacy technician registered and practicing with the GPhC |
| Training and assessment | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>● All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training<br>● All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management<br>● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>● Must work in compliance with the SOPs of their own employer<br>● All users must have appropriate indemnity in place to cover this service |
| Further training and responsibilities | ● All users must be up to date with CPD requirements and appraisal<br>● All users must be up to date with MHRA and safety alerts with regards to medical products<br>● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005<br>● All users must be familiar with Gillick competence and the assessment of consent in children and young people, and with the involvement of a person with parental responsibility |

## Clinical condition or situation to which this PGD applies

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| PGD Indication | Active immunisation of children and young people aged 12 months to 17 years inclusive against travel-related infection, in accordance with Immunisation Against Infectious Disease (the Green Book) and current NaTHNaC / TravelHealthPro country-specific recommendations. Each vaccine has its own minimum age, dose and schedule as set out in the schedule of vaccines table below; the HIGHER of 12 months and the vaccine-specific minimum age applies. |
| Inclusion criteria | • Children and young people aged 12 months to 17 years inclusive, who meet the minimum age for the specific vaccine being administered as set out in the schedule of vaccines table.<br>• Travelling to, or residing in, an area where the vaccine is recommended by NaTHNaC / TravelHealthPro, or otherwise at occupational or lifestyle risk as described in the relevant Green Book chapter.<br>• Valid informed consent obtained from a person with parental responsibility, or from the young person where they are assessed as Gillick competent.<br>• A person with parental responsibility, or a suitable adult authorised by them, is present for the vaccination of a child under 16 years.<br>• Routine UK childhood immunisations are up to date, or a catch-up plan has been discussed and the GP informed. |
| Exclusion criteria | • Aged under 12 months, or below the licensed minimum age for the specific vaccine.<br>• Aged 18 years and over. Use the adult travel health PGDs.<br>• Confirmed anaphylactic reaction to a previous dose of the same vaccine, or to any component of the vaccine, including any excipient or residue listed in the SPC.<br>• Acute severe febrile illness (a minor infection without fever is not a contraindication).<br>• Valid informed consent not obtained, or no person with parental responsibility (or a suitable adult authorised by them) present for a child under 16 years, whether or not the child is Gillick competent.<br>• Significant immunosuppression, including high-dose corticosteroids, biologic or other immunosuppressive therapy, current or recent chemotherapy, asplenia or hyposplenia, or complement deficiency. Refer for specialist advice.<br>• Known or suspected pregnancy in a young person.<br>• Bleeding disorder where intramuscular injection has not been assessed as safe by a clinician familiar with the individual's bleeding risk.<br>• Post-exposure treatment of any kind, including post-exposure rabies management following an animal bite, scratch or lick to broken skin. This requires urgent medical assessment and is outside this PGD.<br>• Complex itinerary, incomplete or unreliable immunisation history that cannot be resolved, or any clinical uncertainty. Refer to the GP or a specialist travel health service. |
| Cautions including any relevant action to be taken | • Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone. |

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| | • Syncope (fainting) is common in older children and adolescents and may occur before or after injection. Vaccinate seated and observe.<br>• Where several vaccines are required, they may be given at the same visit at separate sites, preferably in different limbs, or at least 2.5 cm apart if in the same limb. Record the site of each.<br>• Egg allergy: check the SPC of the specific product. Most travel vaccines in this PGD are not egg-derived, but rabies vaccine presentations differ and must be checked.<br>• Neomycin, polymyxin, streptomycin, latex and thiomersal sensitivities must be checked against the specific product SPC before administration.<br>• Where the child has a chronic condition, is under specialist care, or the itinerary includes prolonged rural travel, remote areas or high-altitude destinations, discuss with or refer to the GP.<br>• Advise that vaccination does not remove the need for bite avoidance, food and water precautions or malaria chemoprophylaxis where indicated. |
| Actions if patient is excluded or declines treatment | Advise on alternative options and how these can be accessed, including the GP practice and specialist travel health clinics, and on the risks associated with travelling unvaccinated. Provide written risk-avoidance advice appropriate to the destination. Document any advice given and the decision reached. Inform or refer to the GP as appropriate. Where post-exposure treatment or urgent assessment is required, arrange this the same day and do not delay. |

## Schedule of vaccines covered by this PGD

*The minimum age, dose and schedule below are taken from the current SPC for each product and the relevant Green Book chapter. Always confirm against the current SPC before administration, as presentations and licensed ages change.*

| Vaccine | Minimum age | Dose and route | Schedule and booster |
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| Hepatitis A (paediatric): Havrix® Junior Monodose, Avaxim® Junior | 12 months to 15 years inclusive (paediatric presentation, licensed to 15 years; a 16 or 17 year old needs the adult presentation under the Hepatitis A and B travel PGD) | 0.5 ml intramuscular | Single dose gives protection from approximately 2 weeks. A second dose 6 to 12 months after the first (Havrix® Junior) or 6 to 36 months after the first (Avaxim® Junior) gives long-term protection. |
| Hepatitis A and B combined (paediatric): Twinrix® Paediatric<br>SmPC: vaccines other than Cervarix should not be given at the same time as Twinrix Paediatric. Give separately, or follow Green Book | 12 months to 15 years inclusive (paediatric presentation, licensed to 15 years; a 16 or 17 year old needs the adult presentation under the | 0.5 ml intramuscular | Three doses at 0, 1 and 6 months. An accelerated schedule may be used where travel is imminent, in accordance with the SPC. |

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| advice and record the reason. | Hepatitis A and B travel PGD) | | |
| Hepatitis A and B combined: Ambirix® | 12 months to 15 years inclusive (paediatric presentation, licensed to 15 years; a 16 or 17 year old needs the adult presentation under the Hepatitis A and B travel PGD) | 1.0 ml intramuscular | Two doses, the second 6 to 12 months after the first. Only suitable where there is a low risk of hepatitis B during the course. |
| Hepatitis B (paediatric): Engerix B® Paediatric, HBvaxPRO® Paediatric | 12 months to 15 years inclusive under this PGD (paediatric presentation, licensed from birth to 15 years; a 16 or 17 year old needs the adult presentation under the Hepatitis A and B travel PGD) | 0.5 ml intramuscular | Three doses at 0, 1 and 6 months, or an accelerated schedule in accordance with the SPC and Green Book Chapter 18. |
| Typhoid (Vi polysaccharide): Typhim Vi®, Typherix® | 2 years | 0.5 ml intramuscular | Single dose at least 2 weeks before travel. Booster every 3 years where exposure continues. |
| Meningococcal ACWY: Nimenrix®, MenQuadfi® | Nimenrix® from 12 months under this PGD (licensed from 6 weeks); MenQuadfi® from 12 months | 0.5 ml intramuscular | Single dose at least 2 weeks before travel. Booster after 5 years where risk continues. Required for pilgrims travelling to Saudi Arabia for Hajj or Umrah. |
| Rabies (pre-exposure): Rabipur®, Verorab® | 12 months under this PGD (licensed at all ages) | 1.0 ml (Rabipur®) or 0.5 ml (Verorab®) intramuscular | Three doses at days 0, 7 and 21 to 28. Pre-exposure only. Any actual or suspected exposure requires urgent medical assessment and is outside this PGD. |
| Japanese encephalitis: Ixiaro® | 12 months under this PGD (licensed from 2 months) | 0.25 ml for children aged 12 months to under 3 years; 0.5 ml for 3 years and over, intramuscular | Two doses 28 days apart. An accelerated schedule is licensed for those aged 18 and over only. |
| Cholera (oral): Dukoral® | 2 years | Oral suspension in buffer | Children aged 2 to under 6 years: three doses at least one week apart. Children aged 6 years and over: two doses at least one week apart. Complete at least one week before travel. |

## Details of the medicines

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| Legal category | POM (all injectable vaccines listed). Dukoral® is a POM oral vaccine. |
| Black triangle | Check the current SPC and MHRA guidance for each product before administration. |
| Off-label use | Any administration outside the licensed age, dose or schedule stated in the SPC is off-label and is outside this PGD, except where explicitly stated in the schedule of vaccines table and supported by the Green Book. Where a vaccine is given off-label in accordance with national guidance, this must be explained as part of the consent process and documented. |
| Storage | Store all vaccines at +2°C to +8°C. Do not freeze. Store in the original packaging to protect from light. Vaccine exposed to conditions outside this range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any decision on continued use. |
| Route/method of administration | ● Administer injectable vaccines by intramuscular injection into the deltoid muscle, or into the anterolateral thigh in younger children where deltoid muscle bulk is insufficient.<br>● Dukoral® is given orally in the supplied buffer, with no food or drink for one hour before and after administration.<br>● Where more than one vaccine is given at the same visit, use separate sites, preferably different limbs, and at least 2.5 cm apart if in the same limb. Record the site of each vaccine.<br>● Inspect each vaccine visually before administration and do not use if the appearance differs from that described in the SPC.<br>● Children with bleeding disorders may be vaccinated intramuscularly where a clinician familiar with the bleeding risk has assessed this as safe, using a fine needle (23 gauge or finer) with firm pressure applied without rubbing for at least 2 minutes. |
| Dose and frequency of administration | As set out in the schedule of vaccines table above, according to the specific product and the age of the child. |
| Quantity to be administered and/or supplied | The doses required to complete the schedule for the vaccines indicated by the pre-travel risk assessment, as set out in the schedule of vaccines table. |
| Disposal | Dispose of used syringes, needles, vials and any discharged vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01. |
| Drug interactions | Vaccines listed may generally be given at the same time as other vaccines at separate sites, EXCEPT Twinrix Paediatric, whose SmPC advises against same-day co-administration with vaccines other than Cervarix (HPV). The immune response may be reduced in children receiving immunosuppressive treatment; such children are excluded from this PGD and require specialist advice. Dukoral® should be separated from oral typhoid vaccine and from antimalarials in accordance with the SPC. Consult the SPC of each product for a full list of interactions. |
| Adverse effects | Very common and common effects across these vaccines include injection site pain, redness and swelling, fever, irritability, headache, malaise, loss of appetite and gastrointestinal upset. Allergic reactions can occur including urticaria, angioedema and, rarely, anaphylaxis. Refer to the SPC and PIL of each product for the full list. |
| Yellow card reporting | Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk. Document any adverse reaction in the child's record and inform their GP. |
| Records to be kept | Record the following information:<br>● that valid informed consent was given, and by whom (see below) |

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| | • name of individual, address, date of birth and GP with whom the individual is registered<br>• name of immuniser<br>• name and brand of vaccine<br>• date of administration<br>• dose, form and route of administration<br>• quantity administered<br>• batch number and expiry date<br>• anatomical site of vaccination<br>• advice given, including advice given if the individual is excluded or declines immunisation<br>• details of any adverse drug reactions and actions taken<br>• that the vaccine was administered via PGD<br>• the destination and travel dates on which the risk assessment was based<br>• confirmation that a person with parental responsibility consented, and their relationship to the child, or the basis of a Gillick competence assessment<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed.<br>• Adults aged 18 years and over: keep records for 8 years<br>• Children aged under 18 years: keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

## Patient information

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| Written information to be given to patient or carer | Offer the marketing authorisation holder's patient information leaflet (PIL) for each vaccine administered. Provide a written record of the vaccines given, with dates, so that the course can be completed elsewhere if necessary, and destination-specific written advice on food and water hygiene, insect bite avoidance, animal avoidance and rabies risk, and malaria prevention where indicated. |
| Follow-up advice to be given to patient or carer | Inform the parent or carer of possible side effects and their management, and advise them to seek medical advice in the event of an adverse reaction and to report it via the Yellow Card scheme. Advise clearly when any further doses are due and that the course should be completed even if travel has taken place. Advise that any animal bite, scratch or lick to broken skin abroad requires immediate wound washing and urgent medical attention regardless of rabies vaccination status, and that any fever during or after travel to a malarial area requires urgent medical assessment. |

## Key references

- Immunisation Against Infectious Disease (the Green Book), Chapters 17 (Hepatitis A), 18 (Hepatitis B), 20 (Japanese encephalitis), 22 (Meningococcal), 27 (Rabies), 33 (Typhoid) and 14a (Cholera)
- NaTHNaC / TravelHealthPro country-specific and vaccine-specific recommendations, checked at the time of each consultation
- Summary of Product Characteristics for each vaccine administered
- UKHSA Vaccine Incident Guidance
- National Minimum Standards and Core Curriculum for Immunisation Training

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions
- NICE MPG2 competency framework for health professionals using patient group directions
- British National Formulary for Children (BNFC)

## Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

| Name and address of pharmacy premises / clinic premises to which this PGD relates | |
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| Name of healthcare professional | Job title | Registration number | Signature |
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| Valid from date | Expiry date |
|---|---|
| 30/9/26 | 31/7/27 |

## PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

*To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.*

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
|---|---|---|---|
| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
| | | | |

## Change history to version 002

### Later versions are recorded in the Version and change record at the end of this document.

| Version number | Change details | Date |
|---|---|---|

| | | |
|---|---|---|
| 001 | Development and issue of new PGD | 30th July 2026 |
| 002 | Adoption and authorisation blocks corrected: those two signatures belong to the adopting pharmacy's superintendent and professional group signatory, not to Get Real Health, and had been pre-filled in error. Clinical content unchanged. | 6th August 2026 |
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**Vaccine safety requirements**

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

**Observation after vaccination**

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

**Version and change record**
Version: v009
Issued: 30 September 2026
Supersedes: Version 008, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

**Previous version records**
Version: v008
Issued: 14 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The exclusion on parental presence now says the same as the inclusion criterion: a person with parental responsibility, or a suitable adult authorised by them, must be present for every child under 16, whether or not the child is Gillick competent.
The records row no longer carries the adult Mental Capacity Act best-interests wording, which cannot apply in a PGD limited to under-18s; consent and by whom is recorded as set out in the same row.
The MenACWY row states the licensed minimum age for Nimenrix (6 weeks), and the rabies row uses the same form of words as the other rows.
The Havrix Junior, Avaxim Junior, Twinrix Paediatric, Ambirix, Engerix B Paediatric and HBvaxPRO Paediatric rows now state that these paediatric presentations are licensed to 15 years inclusive, so a 16 or 17 year old needs the adult presentation under the Hepatitis A and B travel PGD. The previous rows had no upper age in a PGD covering up to 17 years.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity

cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v003

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Observation after vaccination.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.

Version: v005

Issued: 10 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

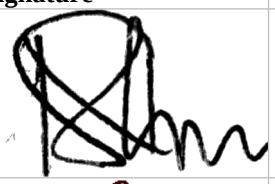
Changes in this version:

Minimum age: the indication said the vaccine-specific minimum age 'always takes precedence over the general lower limit' while the table listed 'from birth' and '2 months', which read literally authorised hepatitis B at birth and Ixiaro at 2 months in a PGD that excludes under 12 months. Now the HIGHER of 12 months and the vaccine-specific minimum applies, and the table says 12 months under this PGD for those rows.

Twinrix Paediatric: the SmPC advises against same-day co-administration with vaccines other than Cervarix. The blanket co-administration statement now carries that caveat.

## Signed on behalf of Get Real Health

Version v009, 30 September 2026.

| Name | Qualification | Signature | Date |
|---|---|---|---|
| Nitin Shori | Medical Director<br>GMC: 6047293 |  | 30 September 2026 |
| Chris Pilkington | Head Pharmacist<br>GPhC: 2046322 |  | 30 September 2026 |

*Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.*